

1. Administrative & Study Identification

Full Study Title: A Phase 2 Open-label Randomized Study of Farletuzumab Ecteribulin (MORAb-202), a Folate Receptor Alpha-targeting Antibody-drug Conjugate, Versus Investigator's Choice Chemotherapy in Women With Platinum-resistant High-grade Serous (HGS) Ovarian, Primary Peritoneal, or Fallopian Tube Cancer **Short Title/Acronym:** A Study of MORAb-202 Versus Investigator's Choice Chemotherapy in Female Participants With Platinum-resistant High-grade Serous (HGS) Ovarian, Primary Peritoneal, or Fallopian Tube Cancer **Protocol Number:** CA116-001 (CA116001) **NCT Number:** NCT05613088 **Posting ID:** 707838954256629772 **Sponsor/Company Name:** Bristol Myers Squibb **Investigational Product:** Farletuzumab Ecteribulin (MORAb-202) **Phase of Development:** Phase 2 **Trial Status:** Completed (Study has been completed) **Medical Monitor:** Clinical Trial Physician – Medical Monitor (Bristol Myers Squibb)

2. Study Synopsis & Rationale

2.1 Study Objective Primary Objective: To assess the safety, tolerability, and efficacy of farletuzumab ecteribulin (MORAb-202) and compare it to Investigator's choice (IC) chemotherapy in female participants with platinum-resistant HGS ovarian, primary peritoneal, or fallopian tube cancer. **Secondary Objectives:** To evaluate overall survival (OS), duration of response (DOR), pharmacokinetic profile, and immunogenicity of MORAb-202, as well as to comprehensively track adverse events (AEs) and serious adverse events (SAEs).

2.2 Background & Rationale To address the significant medical need for effective therapies in women with platinum-resistant HGS ovarian, primary peritoneal, or fallopian tube cancer. Following 1 to 3 prior lines of systemic therapy, patients have limited effective options. This study evaluates MORAb-202, an antibody-drug conjugate targeting Folate Receptor Alpha, which is highly expressed in this tumor population, comparing its clinical benefit against standard single-agent chemotherapies.

3. Study Design & Methodology

3.1 Design Framework Study Type: Interventional **Control Type:** Active Comparator (Investigator's Choice [IC] Chemotherapy: Paclitaxel, Pegylated Liposomal Doxorubicin [PLD], or Topotecan) **Blinding:** Open-label **Randomization:** Randomized

3.2 Planned Enrollment & Duration Target **N**: 106 participants (Targeted/Actual Enrollment) **Number of Sites**: Multicenter global study **Estimated Study Start**: 01-Feb-2023 (Actual) **Estimated Primary Completion**: 17-Jun-2024 (Actual) (*Study Completion: 11-Sep-2025*) **Current Status**: Completed

4. Subject Selection (Eligibility Criteria)

4.1 Inclusion Criteria 1. Female participants with a histologically-confirmed diagnosis of HGS ovarian, primary peritoneal, or fallopian tube cancer. 2. Platinum-resistant disease, defined as progression > 1 month and ≤ 6 months after the last dose of a platinum-based therapy of at least 4 cycles for those with 1 prior line, or progression ≤ 6 months after the last dose for those with 2-3 prior lines. 3. Participants must have received at least 1 but no more than 3 prior lines of systemic therapy and be appropriate for single-agent therapy as the next line. (Participants may have been treated with up to 1 line of therapy subsequent to determination of platinum-resistance). 4. Eastern Cooperative Oncology Group Performance Status (ECOG PS) of 0 or 1. 5. Disease progression per RECIST v1.1 (by investigator assessment) of at least 1 measurable lesion on or after the most recent therapy. 6. Either formalin-fixed, paraffin-embedded (FFPE) tissue (up to 5 years old) or newly-obtained biopsies must be available for FR α assessment prior to randomization.

4.2 Exclusion Criteria 1. Primary platinum-refractory ovarian cancer (defined as disease progression within 1 month of the last dose of the first line platinum-containing regimen). 2. Clear cell, mucinous, endometrioid, or sarcomatous histology, mixed tumors containing components of these histologies, or low grade/borderline ovarian cancer. 3. Pulmonary function test (PFT) abnormalities ($FEV_1 < 70\%$ or $FVC < 60\%$, and $DLCO < 80\%$), investigator-assessed current ILD/pneumonitis, suspected ILD/pneumonitis at screening, or history of ILD/pneumonitis of any severity including from prior anti-cancer therapy. 4. Significant third-space fluid retention (e.g., ascites or pleural effusion) requiring repeated drainage. 5. Evidence of organ dysfunction or clinically-significant deviation from normal physical examination, vital signs, ECG, or clinical laboratory values beyond what is consistent with the target population. 6. Has any prior severe hypersensitivity ($\geq$ Grade 3) to monoclonal antibodies or eribulin, or contraindication to the receipt of corticosteroids or any excipients. 7. History of allergy or contraindication to IC chemotherapy agent selected if randomized to Arm C.

5. Intervention & Treatment Plan

5.1 Investigational Regimen Dosage/Frequency: Dosed per randomization arm; MORAb-202 administered according to protocol dosing schedules, versus Investigator's Choice chemotherapy. IC chemotherapy options include Paclitaxel, Topotecan, or Pegylated Liposomal Doxorubicin (PLD). (Note for PLD - Drug Name: doxorubicin; Trade Names: LipoDox, Caelyx, Doxil, Adriamycin; ATC Code: L01DB01. EMA Information: [Caelyx EPAR](#)) **Route of Administration:** Intravenous (IV). **Duration of Treatment:** Cycles continue until disease progression, unacceptable toxicity, withdrawal of consent, or study completion.

5.2 Concomitant & Prohibited Therapy Permitted: Standard supportive care medications (e.g., antiemetics, analgesics, corticosteroids) per institutional guidelines. **Prohibited:** Concurrent experimental anti-cancer therapies, other systemic antineoplastic therapies, or radiation during the study treatment phase.

6. Study Timeline & Visit Schedule

Visit ID	Window (Days)	Key Activities/Procedures
Screening	Day -28 to 0	Informed Consent, Medical History, Tumor biopsy (FFPE up to 5 yrs old or new), PFTs, RECIST v1.1 Baseline Tumor Assessment.
Baseline	Day 0	Randomization, First Dose.
Interim	Per Treatment Cycle	Efficacy/Tumor assessments (RECIST v1.1), Safety Labs, Vital Signs, AE monitoring (including ILD/pneumonitis screening).
End of Study	Follow-up Phase	Safety follow-up visit (30 days post-last dose), survival follow-up every 3 months up to 1 year from LPLV.

7. Outcome Measures (Endpoints)

7.1 Primary Endpoints 1. **Objective Response Rate (ORR)** by Response Evaluation Criteria in Solid Tumors (RECIST) v1.1 Per Investigator Assessment. 2. **Number of Participants With Treatment-Related Adverse Event (TRAEs) Leading to Discontinuation Within 6 Months From First Dose.**

7.2 Secondary & Exploratory Endpoints 1. Number of Participants With Adverse Events (AEs). 2. Number of Participants With Serious Adverse Events (SAEs). 3. Number of Participants With AEs Leading to Discontinuation. 4. Overall Survival (OS) and Duration of Response (DOR) per RECIST v1.1. 5. Pharmacokinetics (PK) and Immunogenicity incidence rates of MORAb-202.

8. Safety & Adverse Event Reporting

Adverse Event (AE) Monitoring: Continuous recording of AEs via patient interviews, laboratory findings, PFTs, and clinical assessments; special focus on ILD/pneumonitis screening. **Serious Adverse Events (SAEs):** Reported to the sponsor within 24 hours of investigator awareness. **Data Monitoring Committee (DMC):** Utilized to monitor patient safety continuously throughout the trial.

9. Statistical Analysis Plan (SAP) Summary

Analysis Populations: Intent-to-Treat (ITT) for primary efficacy assessments; Safety Set for all participants receiving ≥ 1 dose of study drug. **Sample Size Justification:** Target $N=106$ powered to detect a significant difference in ORR and tolerability between the MORAb-202 arm and the IC chemotherapy arm. **Handling of Missing Data:** Participants dropping out or without an event are censored at the date of last evaluable disease assessment for time-to-event endpoints.

10. Quality Assurance & Regulatory Compliance

GCP Compliance: This study will be conducted in accordance with Good Clinical Practice (GCP) and the Declaration of Helsinki. **Monitoring Plan:** Standard on-site and remote monitoring to ensure protocol adherence, verification of RECIST evaluation, and safety compliance. **Audit Trail:** eCRF tracking, laboratory verification, and comprehensive data clarifications.

11. Study Identifiers & Governance

Primary ID: CA116-001 **Registry Number:** NCT05613088 **Posting ID:** 707838954256629772 **Sponsor/Lead Organization:** Bristol Myers Squibb **Study Title:** A Study of MORAb-202 Versus Investigator's Choice Chemotherapy in Female Participants With Platinum-resistant High-grade Serous (HGS) Ovarian, Primary Peritoneal, or Fallopian Tube Cancer **Official Regulatory Title:** A Phase 2 Open-label Randomized Study of Farletuzumab Ecteribulin (MORAb-202), a Folate Receptor Alpha-targeting Antibody-drug Conjugate, Versus Investigator's Choice Chemotherapy in Women With Platinum-resistant High-grade Serous (HGS) Ovarian, Primary Peritoneal, or Fallopian Tube Cancer

12. Clinical Framework (Oncology Specific)

Disease Areas / Conditions: Neoplasms, Ovarian **Preferred UMLS Name:** Platinum-Resistant Ovarian Cancer (Name/Preferred Name: Neoplasms) **Semantic Type:** Neoplastic Process **MeSH Code:** D009369

SNOMED ID: 108369006 **Diagnosis Threshold:** Histologically confirmed disease with radiologic progression > 1 month and ≤ 6 months following the most recent platinum-based therapy. **Medical Methodology:** Folate Receptor Alpha-targeting Antibody-drug Conjugate versus conventional Chemotherapy (Paclitaxel, PLD, Topotecan). **Optimization Target:** Disease control, Objective Response, and mitigation of disease progression.

13. Study Procedures & Methodology

Management Pathway: Standard oncological care pathway for recurrent/platinum-resistant ovarian cancer. **Education Protocol (HCP):** Site-specific training on ADC safety management (specifically recognizing and managing ILD/pneumonitis). **Education Protocol (Patient):** Informed consent detailing trial procedures, expected treatment toxicities, and the requirement for continuous survival follow-up. **Quality Audit Mechanism:** Continuous safety data reviews, site monitoring, and RECIST v1.1 evaluation verification.

14. Observation & Follow-Up Schedule

Total Duration: Up to 1 year of survival follow-up after the last patient's last treatment visit (LPLV). **Onsite Visit Cadence:** Per protocol-defined chemotherapy/ADC treatment cycles (e.g., every 3-4 weeks depending on the randomized drug schedule). **Remote Monitoring Frequency:** Survival follow-up every 3 months during the post-treatment phase. **Checklist Review Interval:** Routine safety lab assessments and PFT/ECG evaluations at specified intervals during the treatment phase.

15. Sign-off & Approval

Role	Name	Signature	Date				
Principal Investigator	[Insert PI Name]	_____	[DD-MMM-YYYY]				
Clinical Operations Lead	[Insert Name]	_____	[DD-MMM-YYYY]				
Lead Statistician	[Insert Name]	_____	[DD-MMM-YYYY]				